

NCLEX-RN Live Review Tutorial Video

Transcript

INSTRUCTOR: Now, nursing care for the child is different than just the adult. They are not small adults. When we have stress in the hospital—on page 238 and 239, we have a variety of ages. So we have a 2-year-old, a 4-year-old, a 10-year-old, and a 14-year-old. We can provide factual information. And we can do this to more than one. We can encourage self-care. We can give appropriate choices. We can do therapeutic play. So what I want to show you is, all the people that should get factual. If I try to explain something to a 2-year-old or a 4-year-old with facts, 'Well you have an appendix, and it's going to burst if we don't do surgery.' They will have no clue what I'm talking about. But a 10-year-old sure could understand that. We want to encourage them to do the self-care and that they are able to do. But again, a 4-year-old isn't going to give his own bath and brush his teeth without support. Now we do want to make sure everybody has appropriate choices. Because even a 2-year-old can choose to have the red or the blue blanket, if that's what they're looking at. And I know people always say, 'What kind of a therapeutic play are you going to give a 14-year-old?' Well, if they have cancer, you could certainly have a Pac-Man game, where he gobbles up the cancer cells. That would be an opportunity to do therapeutic play. So we can do play with all ages.

Now, very new baby, wouldn't you agree? Kind of cute look. I'd say he's got a pretty good Apgar—he's kind of flexed in, he looks like he's crying, he might have a little acrocyanosis. Everybody's got their gloves on; we're warming him off. But now on this one, we're going to talk about pain. If there is a behavior—we can see grimacing on a baby, we can see crying. There are scales for NICU babies on whether they're in pain. We cannot ask a NICU baby, 'Are you a 10? Are you a 6?' But if it hurts for an adult, it hurts for a baby. Can you tell me: with a circumcision, what are some things that we can do for a baby for discomfort before the circumcision or during? Sugar water on the pacifier, excellent. Because they get the sucrose, and they suckle them, and it's the endorphins that start to circulate. We could also put a numbing cream. One example is EMLA. Has to be put on about an hour before, but it numbs the area so they don't feel it. We also have put it on IV starts, at points. Were you aware that a PCA can be used by a 5-, 6-year-old. They don't have to be able to read. You say, 'Now, when it hurts, you press the button.' Can mommy press the button? No. But we say, 'Mom, if you see him grimacing, or kind of get restless, then say, "oes it hurt? Let's push the button and count to 10 together. One, two, three.'" And by 10, he should get some relief.

Okay. People who are in control of their pain do better. Now, nonanalgesics are great for kids. When I'm giving an injection, don't have a watch—do a song together. They like the song from 'Frozen,' sing the song. The environment. If you have a 4-year-old that needs to have an IV, put him in the red wagon, wheel him down to the treatment room, put the IV in, and wheel them back. Because their bed is their safety environment zone. We can distract them with stories. We can distract them with puzzles. We can distract them with singing. Those are probably not things you're going to do with an adult. You might talk with them while you're getting ready to do a procedure if you've got two people, but you're not going to do it—you're not going to bring an adult out of their bed. They understand, 'Yes, I have to have the IV.' All right, medication administration is different on children.

On to 240, 241, we've talked already about the identification. The child can tell me, and if he says 'I'm Tommie Smith,' then okay. You have to ask the parents for infants. They also have their name band, and some places will have a photo. Because their identifiers have to be a little bit different. When you give an oral med, you've got to make sure that kid can swallow a pill. If he can't swallow a pill, that we need to get it into a liquid format. Also, you don't hand him an 8-ounce glass to take 5 mL of a

medication. You put it in the little medicine cup. Because 5 mL will be easier to get down. Or you can put it in a syringe, without the needle, and squirt it in their mouth. Yeah, don't put the needle on. Which needle size should be used for IM injections? You want to each think: we're looking at how long. Somewhere between 1/2 and 5/8 inch. We might go as big as a 1 for a child, but we want to use the smallest lumen. The smaller the number, the bigger the size. A 14 is a garden hose; a 25 is very small. So we're going to try the smallest that we can.

Now, how are children most likely to respond to terminal illness? Children do die of cancers. There are some pediatric cancers. So we've got an infant, toddler, a preschooler, a school-age child, and a teen. And then we've got a response, senses stressed by the changes in their appearance. It's just one per.

They believe thoughts cause the illness—may be uncooperative for treatment, for fear that they caused it, or something they did or said. And they may mirror the emotions of the parents. For our stressed by changes, it's going to be our teenager. They do not like their hair out of place; they only want the great clothes; and if their friends are going to come visit, make them look better in the hospital. The person that's going to feel that the thoughts caused it is that preschooler. He's only 3 to 6, and he thought about a bunny. And he shooed it out the back and now he thinks because he shooed it out the back—not shot it, but shooed, go away—caused him to have this bad thing happen. Or he hit his brother, now he's being punished. The person who is going to not be cooperative is your school-age child. They can figure some things out, and they may not want to. Because if I don't give you my arm to put the IV in, I won't get one. They don't realize that they have to have it because of the cancer treatment. And the baby—infant/toddler—as long as the parents are calm, they'll do better. So our job is retreating the parents, especially with an infant or a toddler that is having a terminal illness. Very hard, very heartbreaking, when you lose a child.